

The Clinical Conversation - Mastering the Opening with Calgary-Cambridge

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List of contents

- Objectives
- Calgary Cambridge Guideline to the medical interview
- Initiation of the session
- List of complaints
- Agreeing on a common agenda
- Building Rapport

Objectives

- • By the end of this session, you will be able to:
- • Describe the purpose of the Calgary-Cambridge Model.
- • Execute the key steps for effectively initiating the session.
- • Elicit a clear list of the patient's complaints.
- • Negotiate a common agenda for the consultation.
- • Integrate rapport-building throughout the patient interaction.

Why Structure? The Value of a Map

- • • Without a Map:
 - • Missed crucial information.
 - • Patient and clinician frustration.
 - • Inefficient use of time.
 - • Poor therapeutic alliance.

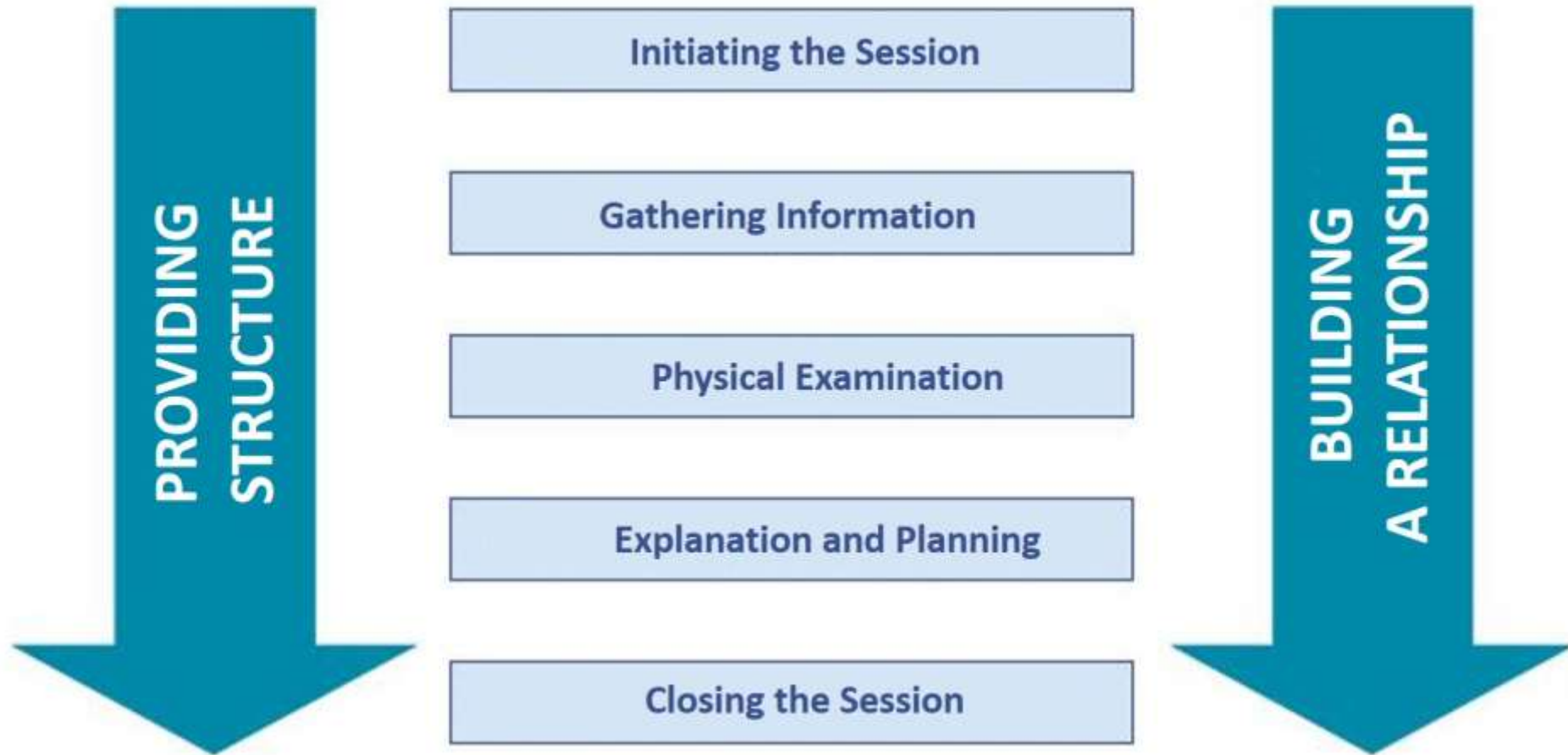
Why Structure? The Value of a Map

- With a Map (Calgary-Cambridge):
 - · Comprehensive data gathering.
 - · Increased patient satisfaction & adherence.
 - · Efficient consultation.
 - · Strong patient-clinician relationship.

The Calgary-Cambridge Model: A Quick Overview

- • • **Initiating the Session**
- • **Gathering Information**
- • **Physical Examination**
- • **Explanation and Planning**
- • **Closing the Session**
- • Running parallel through all stages: Building the Relationship & Structuring the Consultation.

Calgary Cambridge Guideline



Initiation of the Session - The "Golden Minute"

- • Goal: To create a safe, welcoming, and professional environment.
- • Key Tasks:
 - 1. Greet the patient and establish initial rapport.
 - 2. Introduce yourself and identify your role.
 - 3. Identify the patient correctly and ensure comfort & privacy.
 - 4. State the agenda for the session.

Why the Start Matters

- • **First Impressions are Lasting Impressions**
- • • **A Poor Start Can Lead To:**
 - • Patient anxiety and hesitation.
 - • Withholding of important information.
 - • A defensive, non-collaborative atmosphere.
- • **A Strong Start Fosters:**
 - • Patient comfort and trust.
 - • Open, honest communication.
 - • A therapeutic alliance.

The 5 Key Tasks of Initiation

- The 5 Key Tasks
- 1. Greet the Patient & Establish Rapport
 - Action: Warm welcome, smile, non-verbal cues.
- 2. Introduce Yourself & Your Role
 - Action: "Hello, I'm [Name], a medical student with the [Team/Department]."
- 3. Identify the Patient Correctly
 - Action: "You are Mr./Ms. [Surname], is that correct?"
- 4. Ensure Patient Comfort & Privacy
 - Action: Sit down. Close the door/curtain. "Are you comfortable?"
- 5. State the Agenda for the Session
 - Action: "First, I'll listen to your concerns, then we'll do an exam, and then we'll make a plan together."

The Power of Non-Verbals

- Posture: Sit down, lean slightly forward.
- Eye Contact: Maintain it naturally.
- Demeanor: Calm, open, and attentive
- Environment: Private, uninterrupted.

Stating the Agenda: Why It's Crucial

- • Reduces Patient Anxiety: The unknown is scary. A roadmap provides comfort.
- • Manages Expectations: "We may not have time for everything today, but we will address your most pressing concern."
- • Creates Partnership: It invites the patient into the process.
- • Example Phrases:
 - • "To start, I'd like to hear from you about what's been going on. After that, I may have some questions, and then we can discuss what to do next."
 - • "We have about 20 minutes together. My goal is to understand your main concerns today and start a plan."

Do's and Don'ts

- • **DO's (Green Checkmark Column)**
- • Do greet the patient by name.
- • Do sit down and make eye contact.
- • Do state your role and the session's purpose.
- • Do ensure privacy and confidentiality.
- • **DON'Ts (Red "X" Column)**
- • Don't remain standing over the patient.
- • Don't interrupt the patient's first statement.
- • Don't use overly technical language.
- • Don't appear rushed or distracted.

The List of Complaints - Unfolding the Story

- • The Goal: To listen to the patient's full story and identify all of their concerns.
- • The Key Tool: Open-Ended Questions
- • Start with: "What would you like to discuss today?" or "How can I help you?"
- • Then facilitate: "Tell me more about that..." / "Go on..." / Nodding.
- • Active Listening: Listen for content, emotion, and context.
- • Avoid the "Doorhandle Phenomenon": Don't let the patient's main concern pop out just as you're leaving.

Agreeing on a Common Agenda - Becoming a Team

- The Reality: The patient's agenda and your clinical agenda may differ.
- The Process:
 1. SUMMARIZE: "So, from what you've told me, the main things are the knee pain, the trouble sleeping, and feeling stressed. Is that everything?"
 2. PRIORITIZE: "It sounds like the pain is the most pressing issue for you today."
 3. NEGOTIATE: "I suggest we focus on the knee pain in detail today and briefly touch on your sleep. We can schedule a follow-up to address the stress more fully. How does that sound?"

Building Rapport - The Invisible Foundation

- • **What is Rapport? A relationship of trust, understanding, and mutual respect.**
- • **It's NOT a separate step; it's woven into EVERY interaction.**
- • **How to Build It:**
 - • Use the patient's name.
 - • Demonstrate empathy: "I can see how worrying this must be."
 - • Maintain appropriate non-verbal cues: Eye contact, open posture.
 - • Be non-judgmental.
 - • Show support and partnership: "We'll work on this together."

Let's See It In Action: Role-Play Scenario

- • The Patient: Ms. Fatima Muhammad, a 68-year-old woman.
- • The Student: You.
- • The Hidden List:
 - 1. Primary: Dizziness for 2 weeks.
 - 2. Secondary: A worry about her heart (like her sister).
 - 3. Hidden: A request for a refill of her arthritis medication.

Key Takeaways

- • The Calgary-Cambridge Model provides your roadmap for a patient-centered interview.
- • Start strong: A good initiation builds immediate trust.
- • Listen first, diagnose later: Use open-ended questions to get the full story.
- • Be a negotiator, not a dictator: Collaborate on the agenda.
- • Rapport is your greatest tool: Build it with every word and action.

Thank You